Children First, Housing Later*

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Abstract

Should housing assistance require sobriety? A general-equilibrium model of homelessness, drug use, drug prices and mortality is calibrated to US data. Use concentrates among low-income households because a substance use disorder's mortality risk, unlike its unemployment risk, is uninsurable. Drug use harms users' children, cutting their consumption and, through a parent's disorder, their adult earnings. In the long run, sobriety conditions reduce these harms and, in aggregate, also childhood homelessness. Consequently, a newborn's welfare rises by 2.57 percent of lifetime consumption, of which three quarters is an externality its parent does not count, and in general equilibrium parents gain too. Even along the transition, homelessness and childhood homelessness do not rise, and newborns still gain. The model reproduces micro evidence of randomised controlled trials, and users with a disorder lose, but macro effects on other households and children overturn the verdict. Moral hazard alone does not justify conditionality.

Keywords: homelessness; housing vouchers; substance use disorder; intergenerational spillovers; externalities. JEL classification: I18; I38; H23; R21; J62.

1 Introduction

According to the point-in-time (PiT) count of January 2024 performed by the Department of Housing and Urban Development (HUD), at least 771,480 people were sleeping in shelters or on the street, the most since the count began. In July 2025 an executive order directed the federal agencies to end support for Housing First policies that, in its words, "fail to promote treatment, recovery, and self-sufficiency".1 Housing First gives low-income households housing assistance with no requirement to stop using drugs; the alternative, Treatment First, makes the assistance conditional on sobriety and, for those with a substance use disorder (SUD), on being in treatment as well. The funding notice that HUD issued after the order would have capped permanent housing, through which most Housing First assistance is given, at 30 percent of renewal funding, down from more than 80 percent, and would have opened the rest to transitional housing that can require treatment and sobriety, if not for a federal court striking the notice down.2

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the code of the stylized model and extensions of the quantitative model, to debug the code, and to run the simulations. Every number in the paper is generated from that code, the two models were checked against each other on every mechanism they share, with the discrepancies found and their resolution logged in the replication package, and the code is public. The opinions expressed herein do not necessarily represent the views of CMHC.

1Executive Order 14321, Ending Crime and Disorder on America's Streets, 24 July 2025, Federal Register 90, 35817–35820. 2HUD, FY 2025 Continuum of Care Competition and Youth Homeless Demonstration Program Grants, FR-6900-N-25, 13 November 2025; on the shares of funding, Congressional Research Service, Insight IN12626. The ruling is State of Wash- ington v. U.S. Department of Housing and Urban Development, No. 1:25-cv-00626 (District of Rhode Island, 29 June 2026), which set aside both the November and the December 2025 notices. See https://www.npr.org/2025/11/14/nx-s1-5553561/ homelessness-housing-funding-trump-administration-hud; on the court's ruling, https:// marylandmatters.org/2026/06/ 30/ri-federal-judge-strikes-down-huds-2025-restrictions-for-housing-first-initiatives/.

The debate between Housing First and Treatment First is usually framed as a contest between two behavioural claims about the households who receive the assistance.3 Critics of Housing First say that unconditional assistance creates moral hazard: it weakens the incentive to stay sober and employed, so drug use and SUD rise, and with them homelessness. The result is a vicious circle, whose cost falls on others who fund the programme. Critics of Treatment First say that conditionality delays stable housing for those who need it most, because recovering while homeless is harder. In their argument the circle runs the other way, from homelessness to more SUD and back, so Housing First in fact improves the budget. Randomised trials of Housing First speak to both claims: they find large gains in housing stability and mixed-to-null effects on drug use (Section 2). However, they follow the participants for at most three years, and none follows the children into adult life, tracks households outside the trial, or measures the effect on the government's budget, and only one small trial, of homeless adults already in treatment, compares a sobriety condition with unconditional housing (Milby et al., 2005). A model can do all four.

I weigh in on the debate by building a quantitative life-cycle model in which households decide whether to use drugs knowing the risk of acquiring a SUD, and in which drug use harms not only the user but also the user's child. I begin by showing that the moral hazard that dominates the debate does not drive the answer at all: I remove the externalities that harm children, leaving only the change in behaviour and its impact on the budget. A newborn is then worse off in the long run under Treatment First, by 1.09 percent in consumption-equivalent terms, even when drug use and SUD fall substantially and what the drug test saves on vouchers, together with the higher revenues from more employment, is spent on more vouchers for other households. This is because, without externalities, conditionality removes an option from households who weigh the risks of drug use correctly, as the evidence suggests they do (Section 2), and gives them back significantly less through the government's budget than it takes.

I argue that the answer turns instead on the children, through three externalities that a parent takes into account only through imperfect altruism towards the child: two that Housing First leaves in place, and one that many believe Treatment First creates. The two

that Housing First leaves in place fall on the user's children. Children share their parents' consumption and dwelling but not the drug, so every unit a parent spends on drugs, and every unit of earnings lost while a SUD keeps the parent unemployed, is consumption and housing the child goes without: the budget externality. Furthermore, a child whose parent had a SUD while the child lived at home starts adult life with lower productivity: the earnings externality. The one that many believe Treatment First creates runs through homelessness. By denying assistance to drug users, Treatment First exposes more of them to the shelter, and a child lives wherever its parent lives: the childhood homelessness externality. However, I show that, in fact, Treatment First reduces this externality as well.

Whose children bear these costs depends on who uses drugs. Use is concentrated at the bottom of the income distribution: the average user's income is about 74 percent of the population average, and most households do not use at all. The economics literature has reproduced the two facts together only by letting preferences for the drug differ by education (Greenwood et al., 2025). I model drugs as a luxury good bought at a price, which puts most households at a corner where they consume none. On its own, that would concentrate use at the top. What reverses the gradient is the higher risk of dying that comes with a SUD, a loss that rises with income and that, unlike losing one's job, cannot be insured against. The model therefore explains both facts without assuming systematic differences in preferences.

I proceed with two models of rational forward-looking households. The first is a stylized model with two periods of life, in which a SUD raises the risks of dying and of unemployment. Its sole purpose is to show why the income gradient in drug use reverses. The second is a quantitative general-equilibrium life-cycle model calibrated to the United States. Its purpose is to test whether Treatment First raises or lowers welfare relative to Housing First, and whether moral hazard or the externalities on children drive the answer. Its key ingredients are a housing market and homelessness; rationed, means-tested vouchers; income and unemployment risk, with unemployment insurance and food assistance; retirement and social security; recovery and treatment; serious mental illness (SMI); children who share their parent's consumption and dwelling and start adult life with lower productivity if the parent had a SUD during their childhood; a parent's imperfect altruism towards the child; a drug test that misses some users; a drug price that clears

3For the case against Housing First as the federal government now makes it, see the executive order cited above, and on its incentives and costs, Eide (2020); for the case for it, see the National Alliance to End Homelessness, https://endhomelessness. org/blog/ceo-corner-week-of-july-28/, and on housing stability and costs, Tsemberis et al. (2004) and Larimer et al. (2009).

the drug market; and a government budget whose net revenue is held at its benchmark level through the probability of receiving a voucher.

I use the calibrated model to study how the economy changes when it moves from Housing First, the benchmark, to Treatment First. Under Housing First, housing assistance is unconditional for eligible house- holds who draw a voucher. Under Treatment First, it is conditional on not using drugs and, for those with a SUD, on being in treatment as well. Testing is imperfect, however, so some users go undetected and keep their housing

assistance.

The quantitative model lets me separate the three externalities, moral hazard and general-equilibrium effects from one another. I run the same reform with and without the externalities, with only the screen in effect (Treatment First's drug test, which revokes the voucher of a user it catches) and its savings not spent on vouchers, so the one difference between the two runs is whether drug use harms children. Without externalities, with only moral hazard present, a newborn loses 1.17 percent; with them it gains 2.82 percent, childhood included, and the share of children in a shelter actually falls from 1.03 to 0.95 percent. The parents themselves lose about as much whether there are externalities or not. What turns the verdict is what the children gain, which their parents count only partially: more consumption at home, lower homelessness, and adult life without the earnings penalty. In the aggregate, then, there is no trade-off between the externalities: by cutting use, the screen keeps more children out of a shelter than it sends to one by putting their parents out. What the budget, earnings and childhood homelessness externalities have to outweigh is only the direct loss to the parent from the removal of an option. As I explain below, even the parent gains once general- equilibrium effects are taken into account.

Breaking down the gains from Treatment First, I show that spending the screen's savings on more vouchers raises the gain to 6.45 percent. Letting the drug price clear as well, as it would after a real reform, brings the gain down to 2.57 percent, since the lower drug price draws some households back into use. In this full long-run general-equilibrium exercise, Treatment First cuts drug use by 21 percent and the share of households with a SUD by 19 percent.